

TENTATIVE RULINGS 5-19-26
Department R17- Judge Gilbert G. Ochoa

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If you wish to submit on the ruling, call the Court, check-in and state that you will be submitting on the Tentative, and your appearance is not necessary. But you must check in.

If both sides do not appear, the tentative will simply become the ruling.

If any party submits on the tentative, the Court will not alter the tentative and it will become the ruling.

If one party wants to argue, Court will hear argument but will not change the tentative.

If the Court does decide to modify tentative after argument, then a further hearing for oral argument will be reset for both parties to be heard at the same time by the Court.

This procedure is meant to minimize your waiting time in Court.

CIVRS2401232

MICHAEL QUINN

v.

**SAN ANTONIO REGIONAL HOSPITAL, et
al.**

Motion: Motion for Summary Judgment/Adjudication

Movant: Defendant San Antonio Regional Hospital

Respondent: Plaintiff Michael Quinn

DISCUSSION

The Evidentiary Objections

An overview of the law related to expert declarations in medical malpractice cases

“Cases dismissing expert declarations in connection with summary judgment motions do so on the basis that the declarations established that the opinions were either speculative, lacked foundation, or were stated without sufficient certainty. [Citations.] ... It is sufficient, if an expert

declaration establishes the matters relied upon in expressing the opinion, that the opinion rests on matters of a type reasonably relied upon, and the bases for the opinion.” (*Sanchez v. Hillerich & Bradsby Co.* (2002) 104 Cal.App.4th 703, 718.)

However, an expert’s opinion “may not be based on assumptions of facts that are without evidentiary support or based on factors that are speculative or conjectural, for then the opinion has no evidentiary value and does not assist the trier of fact. [Citation.] Moreover, an expert’s opinion rendered without a reasoned explanation of why the underlying facts lead to the ultimate conclusion has no evidentiary value because an expert opinion is worth no more than the reasons and facts on which it is based.” (*Powell v. Kleinman* (2007) 151 Cal.App.4th 112, 123, citing *Bushling v. Fremont Medical Center* (2004) 117 Cal.App.4th 493, 510.)

Although an expert can rely upon hearsay in forming an opinion, the expert must show personal knowledge of the facts upon which the opinion is based unless the medical records are properly authenticated, such as “by means of a declaration or deposition testimony from the doctor who performed the surgery, or by properly authenticated medical records placed before the trial court under the business records exception to the hearsay rule.” (*Garibay v. Hemmat* (2008) 161 Cal.App.4th 735, 738.)

Plaintiff’s objections

In *San Antonio Regional Hospital v. Superior Court* (2024) 102 Cal.App.5th 346, 348-349, the plaintiff’s adult son died while being treated by SARH for a traumatic brain injury. In the suit that followed, the plaintiff claimed SARH was negligent because surgical intervention was “belatedly performed” and the nursing staff failed to adequately monitor the decedent, inform his doctor about his status, and advocate for the need for earlier surgical intervention. SARH moved for summary judgment and submitted two declarations from doctors indicating SARH provided

adequate care. Plaintiff opposed the motion with a declaration from a nurse anesthetist, who opined that there was a delay in providing the medical intervention, in contacting the decedent's physician, and in implementing treatment modalities, which were substantial factors in causing or contributing to the death.

The trial court in *San Antonio Regional Hospital* denied SARH's motion based on the nurse's declaration. On writ review, the court of appeal reversed and concluded the trial court erred since the plaintiff's only expert, the nurse, lacked the requisite skill or experience to opine on the standard of care and causation. The court of appeal noted that while "[q]ualifications other than a license to practice medicine may serve to qualify a witness to give a medical opinion," there still "must be some aspect of the expert's qualifications or experience to show the expert has competencies "beyond common experience" that bear on the relevant factual questions." (*San Antonio Regional Hospital v. Superior Court* (2024) 102 Cal.App.5th 346, 352 [the court provided the example of a toxicologist who, while not a doctor, was able to opine that a murder victim died of poisoning by an agricultural chemical].)

The nurse before the court in *San Antonio Regional Hospital* had not established that she had specialized knowledge applicable to an intensive care unit neurosurgeon deciding whether a traumatic brain injury requires surgical intervention. The nurse also failed to establish competence to opine on the issue of causation because the perspective of the doctor is required to opine as to the role that nurse communications played in the doctor's decision-making process and as to whether the decision to operate earlier would have likely led to a different outcome. (*San Antonio Regional Hospital, supra*, 102 Cal.App.5th at p. 353 [though noting the nurse could opine as to as when and how a nurse should communicate with a doctor regarding patient care].)

In this case, Michael objects to the declaration from Ransbury, but he only specifically argues that her opinion as to causation lacks foundation based on the holding in *San Antonio Regional Hospital*. Ransbury establishes herself as an expert related to wound care and the standard of care for nurses in the hospital setting. As a result, Ransbury would be qualified to opine as to whether SARH complied with the standard of care. In that regard, the objection to the “entire” declaration from Ransbury should be overruled. (See *OCFCD v. Sunny Crest Dairy, Inc.* (1978) 77 Cal.App.3d 742, 753 [To the extent the material cited as being objectionable contains any statement that is non-objectionable, even if meritorious objections could have been posed to unspecified portions of the material, the objection should be overruled].)

Even if the objection were specific to, and if it expressly referenced, paragraph 36 of the Ransbury declaration (which consists of the opinion on the lack of causation), Ransbury provided the basis for her opinion, namely, the decedent was refusing care, she had the ability to make her needs known and to “micro-reposition,” and she was taking Levophed which decreased peripheral circulation and rendered repositioning “almost pointless.” The opinion is thus based upon facts within Ransbury’s realm of expertise. The objection is therefore be overruled.

Defendant’s objections

Notably, in ruling on Kindred’s prior summary judgment motion, the Court sustained Kindred’s objections to the Holman declaration related to the CDPH materials and prior investigations. (See 4/21/26 Ruling [noting that CDHP deficiency citations and investigation materials did not fall within the exception to the hearsay rule and, regardless, there is no nexus between the unrelated deficiency findings and the care of the Decedent].)

The same logic would apply as to SARH’s objections to the Holman declaration as it also relates to the CDPH investigations and the opinions based upon the investigations in unrelated

matters. Furthermore, while it is common for an expert to rely upon medical records in forming an opinion, Michael has not necessarily shown it is common for an expert to rely upon an investigation report that in turn relies upon other hearsay or medical records.

As a result, the Court overrules objection no.'s 1 and 2 (as overbroad, see *OCFCD, supra*, 77 Cal.App.3d at p. 753) and 18 to the Holman declaration, but sustain objection no.'s 3-17 and 19-20 to paragraphs 25-38, 40, and 45- 47 of the Holman declaration.

The declaration from Korbaj also similarly references and at times relies upon the CDPH investigations and, therefore, the same analysis the Court previously adopted would apply. On the other hand, Korbaj can appropriately opine as to the level of care and causation for purposes of the malpractice and wrongful death claims. As a result, the Court overrules objection no.'s 1-10, 14-16, 18-20, and 23-24 to the Korbaj declaration (again some of the objections are simply too overbroad), but sustain objection no.'s 11, 12, 13, 17, 21, 22, and 25.

The Second and Third Causes of Action for Malpractice and Wrongful Death

An overview of the law

The elements of a cause of action for medical malpractice are ““(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional’s negligence.” (*Gami v. Mullikin Medical Center* (1993) 18 Cal.App.4th 870, 877 [citing *Budd v. Nixen* (1971) 6 Cal.3d 195, 200].)

A claim for wrongful death can be premised upon a viable malpractice claim. The elements of a wrongful death claim are (1) a wrongful act or neglect on the part of one or more persons that (2) causes (3) the death of another person. (*Norgart v. Upjohn Co.* (1999) 21 Cal.4th 383, 390.)

In medical malpractice cases, “expert opinion testimony is required to prove or disprove that the defendant performed in accordance with the prevailing standard of care except in cases where the negligence is obvious to laymen.” (*Kelley v. Trunk* (1998) 66 Cal.App.4th 519, 523 citing *Miller v. Los Angeles County Flood Control Dist.* (1973) 8 Cal.3d 689, 702.) Expert testimony is admissible if it is based on “matter of a type that may reasonably be relied on by an expert in forming an opinion on the subject to which his testimony relates.” (*Ibid.*) The reason an expert is needed is because the standard of skill, knowledge and care prevailing in a medical community is ordinarily a matter within the knowledge of experts. (*Jambazian v. Borden* (1994) 25 Cal.App.4th 836, 844.)

In fact, “[w]hen a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence.” (*Munro v. Regents of the University of California* (1989) 215 Cal.App.3d 977, 955.)

An expert’s opinion that no breach occurred or no causation exists is sufficient to support summary judgment where the plaintiff fails to submit any opposing expert opinion. (*Jambazian v. Borden* (1994) 25 Cal.App.4th 836, 850.) However, “an opinion unsupported by reasons or explanations does not establish the absence of a material fact issue for trial, as required for summary judgment.” (*Kelley v. Trunk* (1998) 66 Cal.App.4th 519, 524.)

Here, and as was the case related to Kindred’s motion, there are triable issues of material facts as to whether SARH’s conduct fell below the standard of care and whether that alleged negligence was a causal factor in the injuries claimed and (ultimately) Decedent’s death. The disputed material facts arise from the conflicting expert declarations. (Compare Ransbury Decl. at ¶¶ 30 and 36 to Korbaj Decl. at ¶¶ 12, 15, 33, 35, 50, 57, and 66; Fact No.’s 56 and 63 and the

Responses). The motion for summary judgment and the motion for summary adjudication, as to what the notice of motion identifies as “issue” number 5, is therefore denied.

The First Cause of Action for Elder/Dependent Adult Abuse

An overview of the applicable law

The abuse of an elder/dependent adult under the Elder Abuse and Dependent Adult Civil Protection Act (“Act”) includes neglect. (Welf. & Inst. Code, § 15610.07, subd. (a)(1).) Neglect means the “negligent failure of any person having the care or custody of an elder or dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.” (Welf. & Inst. Code, §15610.57, subd. (a)(1).) Neglect can include the failure to assist in personal hygiene or provision of food, clothing, or shelter; failure to provide medical care for physical and mental health needs; failure to protect from health and safety hazards; and failure to prevent malnutrition or dehydration. (Welf. & Inst. Code, § 15610.57, subd. (b)(1)-(4).)

A neglect dependent or elder adult claim requires: (1) that the defendant had care or custody of the plaintiff, (2) that the plaintiff was an elder or dependent adult while in the defendant’s care or custody, (3) that the defendant failed to use the degree of care that a reasonable person in the same situation would have used, (4) that the plaintiff was harmed, and (5) that the defendant’s conduct was a substantial factor in causing the plaintiff’s harm. (CACI 3103; *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, 406-407 (*Carter*).) Elder abuse claims must be pled with particularity. (*Covenant Care, Inc. v. Superior Court (Inclan)* (2004) 32 Cal.4th 771, 790 (*Covenant Care*); *Carter, supra*, 198 Cal.App.4th at p. 407.)

SARH’s motion

As a preliminary matter, it should be noted that summary adjudication is not sought as to any specific cause of action per se. Instead, the motion outlines the “grounds” for summary

judgment and then seeks in the alternative summary adjudication of “issues” as opposed to summary adjudication of each cause of action. Where summary adjudication is sought, the notice must specify the “specific cause of action, affirmative defense, claims for damages, or issues of duty” sought to be adjudicated.” (Cal. Prac. Guide Civ. Pro. Before Trial Ch. 10-C [citing Cal. Rules Ct., rule 3.1350(b)].)

Summary adjudication is also only proper if it completely disposes of a “cause of action, an affirmative defense, a claim for damages, or an issue of duty.” (Code Civ. Proc., § 437c, subd. (f)(1).) The “issues” raised in the motion in this case are not “issues of duty.” Issue no. ’s 2-4 would also not completely dispose of the claim because each is an alternative basis for liability. As a result, only “issue” no. 1 would completely dispose of the abuse claim or at least the claim for enhanced remedies.

In any event, and as to the substantive arguments advanced, the motion outlines the care that SARH provided to Decedent, which, as a whole, largely could suggest that SARH did not neglect Decedent. However, Michael validly argues that providing some care at some times and in some areas does not, in and of itself, mean there was no neglect. If nothing else, the opposing facts and evidence indicates there was repeated failure to implement basic preventative measures such that it constitutes reckless conduct. (Response to Fact No. 30.)

The opposing evidence could also suggest that neglect occurred related to Decedent’s nutritional needs because on September 22, 2023, there was a GI consultation, yet she was diagnosed with severe malnutrition on September 8. While the family and Decedent indicated they would consider the G-tube, there is no explanation for the initial delay. Furthermore, during the subsequent admission in December 2023, Decedent did not receive a feeding tube timely and it was also not properly administered. Thus, Michael contends Decedent’s nutritional needs were

neglected for about a week after the second admission. (See Response to Fact No.'s 10 and 36-39.) It should also be noted that the motion is heavily dependent upon citations to the medical records, which again are not necessarily the easiest to understand (in terms of what they show or their significance), thus suggesting expert guidance is needed, but Dr. Korbaj concludes that the conduct outlined within the records shows custodial neglect. (Dr. Korbaj Decl. at ¶ 66.)

In the motion, SARH also attempts to address the absence of corporate liability. It primarily relies upon the law related to the recovery of *enhanced* remedies. A Plaintiff seeking the elder/dependent adult abuse statute's enhanced remedies against an employer for the acts of its employees must prove by admissible, clear and convincing evidence, that: (1) defendant's employees neglected a elder/dependent adult; (2) the employer acted with oppression, fraud, or malice; and (3) an officer, director or managing agent of the employer had advanced knowledge of the unfitness of the employee and employed him or her with a conscious disregard of the rights or safety of other or authorized or ratified the wrongful conduct for which the damages are awarded or was personally guilty of oppression, fraud or malice. (Welf. & Inst. Code, § 15657, subd. (c).)

Notably, however, such a showing is only required for the "enhanced remedies," but not to otherwise prevail on a cause of action for elder abuse. (Compare CACI 3102A and 3102B [elements for employer liability for enhanced remedies] and CACI 3103 [outlining the general elements of a claim for neglect under the same statutory scheme].) In this regard, the absence of recklessness, conscious disregard, etc., would not completely dispose of the cause of action as a whole, though it would undermine any claim for enhanced remedies.

Regardless, and as with Kindred's motion, Michael and his experts attempt to rely on the CDPH report and investigation to establish the corporate liability, but as noted above SARH asserted valid objections to that aspect of the evidence. Nevertheless, it was SARH's duty as the

moving party to disprove the corporate liability. The declarations from Chapman and Dhillon are insufficient if for no other reason than the fact that liability could be based upon conduct of some other office director or managing agent, i.e., SARH presents a strawman argument of sorts and does not demonstrate that no officer, director, or managing agent did not have the requisite advanced knowledge, etc.

Even if that were not the case, SARH only addresses whether Chapman and Dhillon participated in the bedside care or supervision of patients and that the two did not hire those responsible for that care. The evidence also indicates neither “implement” such care. If nothing else, the declarations do not establish that Chapman and Dhillon were unaware of what purportedly was occurring and thus their failure to act in the face of such knowledge could support liability.

Ruling

The Court rules as follows:

- (1) Overrule Plaintiff’s objection to the Ransbury declaration.
- (2) Overrule SARH’s objection no.’s 1 and 2 (as overbroad) and 18 to the Holman declaration, but sustain objection no.’s 3-17 and 19-20 to paragraphs 25-38, 40, and 45-47 of the Holman declaration.
- (3) Deny the motion for summary judgment.
- (4) Deny the motion for summary adjudication as to what is identified as “issue” no. 5 in the notice of motion (related to the second and third causes of action).
 - a. There is a triable issue of material facts as to whether SARH’s conduct fell below the standard of care and whether that alleged negligence was a causal factor in the injuries claimed and (ultimately) Decedent’s death. (Compare Ransbury Decl. at

¶¶ 30 and 36 to Korbaj Decl. at ¶¶ 12, 15, 33, 35, 50, 57, and 66; Fact No.'s 56 and 63 and Responses).

(5) Deny the motion for summary adjudication as to issue no.'s 1-4.

- a. The motion is procedurally defective in that summary adjudication is not directly sought as to the causes of action, the “issues” outlined are not issues of duty, and issue no.'s 2-4 would not completely dispose of the cause of action independently. (See Cal. Rules Ct., rule 3.1350(b); Code Civ. Proc., § 437c, subd. (f)(1).)
- b. There is also a triable issue of material fact as to whether neglect under the act occurred. (See, e.g., Response to Fact No.'s 10, 30, and 36-39; Dr. Korbaj Decl. at ¶ 66.) The burden outlined by SARH also addresses the “enhanced” remedies, but the underlying claim is still a cause of action for neglect, though there is some uncertainty on this issue. (Compare CACI 3102A and 3102B [element for employer liability for enhanced remedies] and CACI 3103.)
- c. SARH also failed to show an absence of corporate liability and therefore failed to meet its initial burden. For instance, it is not clear that the claim was merely based upon Chapman's and Dhillon's role and in this regard their declarations present a strawman argument of sorts. The two also do not refute having knowledge of the purported neglect of Decedent and such knowledge, coupled with a failure to act, could support liability.

Movant to give notice.

Dated-

Judge